

Therapist Differences in Symptom Change With Racial/Ethnic Minority Clients

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Some psychotherapists produce better outcomes than others (Crits-Christoph & Mintz, 1991; Kraus, Castonguay, Boswell, Nordberg, & Hayes, 2011), although it has not been established whether therapists vary in their effectiveness with racial/ethnic minority (REM) clients. The importance of identifying therapist factors associated with successful outcomes for REM clients is underscored by imperatives regarding the provision of culturally competent psychotherapy (American Psychological Association, 2003; Smith, Rodriguez, & Bernal, 2011) and by research demonstrating the existence of ethnic disparities in mental health problems (Hayes, Chun-Kennedy, Edens, & Locke, 2011a) and their treatment (Harris, Edlund, & Larson, 2005). In this study, we investigated 36 therapists and 228 clients seen at a university training clinic to investigate whether differences in therapist effectiveness were a function of client ethnicity. Clients completed the Outcome Questionnaire-45 prior to each session. Multilevel modeling analyses indicated that (a) outcomes for REM and non-REM clients did not differ, (b) some therapists produced better outcomes than others, and (c) this variability was due in part to client REM status. Thus, it appears that therapists vary in their effectiveness at reducing symptoms with REM clients. Implications for training, multicultural theory, and future research are discussed.

Keywords: therapist effects, psychotherapy outcomes, multicultural

Research has established that some psychotherapists produce better outcomes than others (Crits-Christoph & Mintz, 1991; Kim, Wampold, & Bolt, 2006; Kraus, Castonguay, Boswell, Nordberg, & Hayes, 2011), with particularly effective therapists having been referred to by some as “super shrinks” (Okiishi, Lambert, Nielsen, & Ogles, 2003). Empirical evidence has only recently begun to test whether therapists vary in their effectiveness with racial/ethnic minority (REM) clients.¹ The importance of identifying therapist factors associated with successful outcomes for REM clients is underscored by ethical imperatives regarding the provision of culturally competent psychotherapy (American Psychological Association, 2003; Smith, Rodriguez, & Bernal, 2011) and by research demonstrating the existence of ethnic disparities in mental health problems (Hayes, Chun-Kennedy, Edens, & Locke, 2011a) and their treatment (Harris, Edlund, & Larson, 2005; U.S. Surgeon General, 2001). According to Meyer’s (2003) minority stress theory, members of cultural minority

groups face a number of sociopolitical difficulties, such as discrimination, oppression, and prejudice, which adversely affect mental health. At the same time, some data suggest that REM children develop protective mechanisms to deal with these external forces and, as a result, manifest fewer internalized mental health problems as adults than do European Americans (Breslau, Auilar-Gaxiola, Kendler, Su, Williams, & Kessler, 2006).

Further complicating matters, when REM individuals do experience mental health problems, they may be hesitant to seek treatment from European American therapists due to factors such as cultural mistrust, concerns about misdiagnosis, doubts about the availability of culturally sensitive services, and cultural norms related to self-reliance and privacy (Barksdale & Molock, 2009; Duncan, 2003; Hayes et al., 2011b; Ward & Besson, 2013; Whaley, 2001). When REM individuals do receive psychotherapy, research suggests that it may be difficult for some clients to form an alliance with their therapists (Flückiger et al., 2013) and that they may be at increased risk for premature termination (Kearney, Draper, & Baron, 2005; Terrell & Terrell, 1984; Wade & Bernstein, 1991), although this finding has not been replicated uniformly (Maramba & Hall, 2002; Shin et al., 2005).

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¹ The term “REM” refers to individuals who identify their race or ethnicity as other than White, European American, or Caucasian.

Therapists' Cultural Competence

Therapists' competence in delivering culturally effective services has been a topic of interest for >20 years (Sue, Arredondo, & McDavis, 1992). This interest has produced a rather large volume of theoretical writings, suggestions for training, and clinical admonitions (Ponterotto, 2008), though a comparatively limited amount of research. Much of the research that does exist on therapist cultural competence relies on self-report measures (e.g., of therapists' cultural awareness, skills, and knowledge), which are susceptible to distortion and tend not to converge with observers' ratings of therapists' cultural competence (Worthington, Mobley, Franks, & Tan, 2000). Even when clients have attempted to rate their therapists' cultural competence, client ratings tend to be similar across therapists and do not consistently predict therapy outcome (Larrison, Schoppelrey, Hack-Ritzo, & Korr, 2011; Owen, Leach, Wampold, & Rodolfa, 2011; Worthington, Soth-McNett, & Moreno, 2007). Wampold and Brown (2005) argued convincingly that therapists' competence ultimately should be determined by the extent to which their clients improve in therapy. By extension, therapist cultural competence is best defined in terms of the improvement shown by their REM clients.

Recent Multilevel Modeling Research on Therapists' Cultural Competence

Some authors have questioned whether cultural competence is a component of general therapist competence or whether it is a specific therapist factor (Coleman, 1998; Imel et al., 2011). If cultural competence was not distinct from general competence, then therapists who produced good outcomes with White clients also would produce good outcomes with REM clients, and therapists who produced poor outcomes with White clients also would produce poor outcomes with REM clients. In other words, client REM status would not be associated with therapist effectiveness. If, however, therapist cultural competence can be distinguished from general therapist competence, then some therapists will produce better outcomes with REM clients than other therapists, but these therapists will not produce better outcomes across all clients. These therapists could be thought of as "cultural experts," and they might serve as models to study, learn from, and emulate in terms of producing good outcomes with REM clients. They might be particularly instructive for those therapists who produce good outcomes with White clients but not with REM clients, as presumably such therapists lack cultural competence. Finally, there is the case of the so-called "pseudoshrink," those 5% of therapists who consistently produce poor outcomes across most of their clients (Kraus et al., 2011) and who seem to lack both general and cultural competence.

One study on therapist effects with REM clients (Imel et al., 2011) found that some of the 13 therapists in their sample were more effective with REM clients and others were more effective with White clients, providing further evidence of possible therapist effects in the provision of services to REM clients. Outcome in the Imel et al. study, however, was limited to cannabis use in an adolescent sample, leaving open the more general question about differential therapist effectiveness in reducing psychological symptoms with adult REM clients. Toward this end, Larrison et al. (2011) examined the outcomes of 62 therapists working in 13

community mental health centers with 551 clients, 138 of whom were Black and the remainder of whom were White. Outcome was measured in terms of changes on the Behavior and Symptom Identification Scale. Multilevel modeling analyses revealed that the relationship between client race and outcome was moderated by the therapist with whom the client worked. In particular, nearly a fifth of the therapists had better outcomes for White clients than Black clients, and 39% of therapists had better outcomes for Black clients than White clients. Unfortunately, some therapists in the study worked with as few as one client of either race which undermines the robustness of the findings. In a related study of differential therapist effectiveness with REM clients, Owen, Imel, Adelson, and Rodolfa (2012) found that therapists varied in the percentage of REM clients who prematurely terminated from counseling, although similar results were not reported in a study by Larrison and Schoppelrey (2011).

In the present study, we sought to examine symptom reduction as one important component of defining cultural competence. By investigating changes in amount of symptom reduction, we hoped to add to the small body of empirical literature examining therapist effects in terms of the outcomes of services to REM clients. We hypothesized that therapists would differ in the amount of symptom change they evidenced with their clients, and that these differences would be associated with clients' REM status.

Method

Participants

Archival records from a university training clinic at a mid-Atlantic university from 2004 to 2011 provided the data for this study. During that 7-year period, 253 graduate student trainee therapists saw 1,120 clients for a maximum of either one semester (for students in counselor education) or two semesters (for students in counseling psychology). From that data set, we examined only those therapists who saw at least two REM clients and at least two White clients.

The final data set consisted of 36 therapists and 228 clients, 80 of whom were REM individuals. The client sample had the following characteristics: 79.4% women and 20.6% men; 10.1% African American, 10.1% Asian American, 7.9% Hispanic/Latino/a, 2.6% multiethnic, 64.9% European American, and 1.3% other; 15.8% first-year students, 20.2% sophomores, 32.9% juniors, 25.4% seniors, 5.3% graduate students, and 0.4% other; and a mean age of 20.74 years ($SD = 2.75$). Although formal diagnoses were not provided for clients, two of the authors provided clinical supervision to trainees and served as directors of the clinic during the time data were collected, and on this basis it was known that typical client presenting concerns included depression, anxiety, relationship issues, and academic distress. Clients were seen for an average of 5.42 sessions.

Therapists saw between 4 and 13 clients (mode and median = 6). Therapists were doctoral and master's students in counselor education and doctoral students in counseling psychology. Of the 36 therapists, 32 were women and 34 were White.

Measures

The Outcome Questionnaire-45 (OQ-45) is a client self-report instrument designed to measure psychological distress (Lambert et

al., 1996; Lambert & Shimokawa, 2011). Items are scored on a 5-point Likert-type scale (0 = *never*, 1 = *rarely*, 2 = *sometimes*, 3 = *frequently*, and 4 = *almost always*). Higher scores reflect greater distress. The OQ-45 is a widely used instrument with internal consistency estimates $>.90$, evidence of construct validity via correlations with measures such as the Beck Depression Inventory and Symptom Checklist-90, and demonstrated sensitivity to change in psychotherapy (Lambert et al., 1996; Okiishi et al., 2003).

Procedures

Clients were either referred from the university counseling center after an initial intake interview or self-referred from undergraduate classes where clinic services were advertised. Clients were screened out for services if they were suicidal, in need of psychotropic medication, likely to be in crisis, in need of more than one session per week, or unwilling to have their sessions videotaped for supervisory purposes. Clients were assigned to therapists by a team of clinic supervisors (two advanced doctoral students and a supervising faculty member) according to fit with clients' schedules, number of current clients on therapists' caseload, severity of client issues, and therapist experience. Therapists received one hour per week of individual supervision. Doctoral students were supervised by department faculty members, and master's students were supervised by advanced doctoral students whose supervision was in turn supervised by department faculty members. Specific approaches to both therapy and supervision varied according to client needs, individual therapists, and their supervisors. No predominant approaches to therapy or supervision characterized the training clinic where the study was conducted. Clients filled out a demographic questionnaire prior to the first session and completed the OQ-45 prior to each counseling session as a routine part of clinical services at the training clinic.

Data Analysis Considerations

Given the nested nature of the data, we used multilevel models to correct for possible interdependencies in the data (i.e., multiple clients treated by the same therapist). Multilevel models with Bayesian estimation were analyzed using the statistical package Mplus 7.0. Bayesian models are not based on normality assumptions or asymptotic results and provide better estimations for unbalanced and relatively small sample sizes (Hamaker & Klugkist, 2011), which is advantageous for this study, which had a moderate number of therapists and an unbalanced number of clients treated by each therapist. Bayesian models do not typically use traditional significance tests. Rather, in Bayesian models, the results describe posterior distribution, which is the range of uncertainty left in the model after accounting for data included in the model (Hamaker & Klugkist, 2011). Thus, larger posterior distributions mean that there is more uncertainty in the results. Credible intervals (CIs), similar to confidence intervals, are used in Bayesian models to help describe the range of the posterior distribution. Our approach is consistent with previous studies examining therapists' variability in their effectiveness with REM clients (Imel et al., 2011; Owen et al., 2012).

Specifically, we conducted a two-level model, wherein clients (Level 1) were nested within therapists (Level 2). Our first model

tested whether client race/ethnicity was a credible predictor of post OQ-45 scores after controlling for OQ-45 pre scores and therapist race/ethnicity. The combined question for Model 1 (titled Client RE Status Fixed) was

$$OQ45_{post} = y_{00} + y_{01}(TherapistRE) + y_{10}(OQ45_{pre}) + y_{20}(ClientRE) + u_0 + r.$$

Next, we tested whether the association between client race/ethnicity and OQ-45 post scores varied across therapists. Statistically speaking, the only difference between the two models rests with the random effect for the association between client race/ethnicity and outcome at Level 2, which is bolded in the equation below for Model 2 (titled Client RE Status Random):

$$Q45_{post} = y_{00} + y_{01}(TherapistRE) + y_{10}(OQ45_{pre}) + y_{20}(ClientRE) + u_0 + u_2(ClientRE) + r.$$

Conceptually, the primary difference between the two models lies with the decision to treat the association between client race/ethnicity as fixed or random at the therapist level. Fixing the effects across therapists assumes that the association between client race/ethnicity and outcome is consistent for all therapists, whereas treating the association between client race/ethnicity and outcome as random allows for each therapist to have their own unique regression coefficients for these associations. The random effect will address whether some therapists are generally better or worse at facilitating change with REM or White clients (e.g., therapist-specific racial/ethnic disparity). It is important to note that nearly all of the therapists identified as White (34 of 36), so we were not able to accurately model the cross-level interactions in these models and we only utilized therapist race/ethnicity as a control variable.

Results

Table 1 provides an overview of the outcome data for REM and White clients. Notably, ~31% of both REM and White clients met criteria for reliable change based on Jacobson and Truax's (1991) criteria. The two groups were similar in the percentage of clients who reported deterioration and no change. To test our hypotheses,

Table 1
Descriptive Information for Clients' OQ-45 Scores

	REM clients <i>M</i> (<i>SD</i>)	White clients <i>M</i> (<i>SD</i>)
Initial OQ-45 scores	61.35 (24.12)	56.07 (20.43)
Final OQ-45 scores	51.93 (26.64)	47.43 (23.42)
Number of sessions	5.40 (3.77)	5.43 (3.99)
	<i>n</i> (%)	<i>n</i> (%)
Improved	25 (31.3)	47 (31.8)
No change	52 (65.0)	93 (62.8)
Deteriorated	3 (3.8)	8 (5.4)

Note. OQ-45 = Outcome Questionnaire-45. The numbers of clients who improved, did not change, or deteriorated were based on reliable change scores for the OQ-45 (e.g., those who improved more than the reliable change index of 14 points from pre- to posttreatment).

we ran the two models described above. The results from the first model demonstrated that there were no credible differences between REM and White clients in posttreatment OQ-45 scores. In other words, there were no general mental health disparities in therapy outcomes (see Table 2). Examining the variance components, therapists accounted for 8.7% of the variance in therapy outcomes in this first model (i.e., therapist variance or Level 2 variance/total variance). The results from the second model revealed that the therapist-level random effect for the association between client race/ethnicity and OQ-45 posttreatment scores was credible, but the credible interval was wide, suggesting less certainty in this effect.

In more concrete terms, we examined the reduction in variance at the therapist level when client race/ethnicity was allowed to vary across therapists, as compared with Model 1 where client race/ethnicity was treated as fixed across therapists. Specifically, therapist variance in outcomes for Model 1 (client race/ethnicity was fixed) was 22.99, whereas it was 18.59 in Model 2 (client race/ethnicity was random). Thus, client race/ethnicity treated as random effect across therapists explained 19.1% of the variance (4.40/22.99) in treatment outcomes attributed to therapists. In other words, some therapists produced better outcomes than other therapists, and this variability was a partial function of client REM status.

We randomly selected 10 therapists to illustrate the differences in pre–post change scores (see Figure 1). For example, Therapist 9 was generally ineffective at treating both White and REM clients, whereas Therapist 2 demonstrated ~14 point changes for his/her White and REM clients. There were disparities for some therapists as a result of clients' REM status. For example, Therapist 3 had better outcomes with his/her White clients compared with their REM clients. The opposite pattern can be seen for Therapist 5, for example.

Discussion

The findings from this study add to the small growing body of empirical literature on therapist effects with REM clients. Similar to Owen et al. (2012), who found that therapists differed in their drop-out rates among REM clients, as well as Larrison et al.

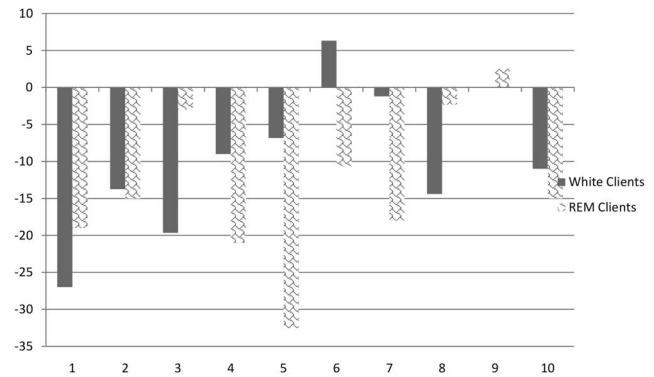


Figure 1. REM and White clients' OQ-45 change scores for 10 randomly selected therapists. OQ-45 = Outcome Questionnaire-45.

(2011), who found therapist effects associated with client race in reducing psychological symptoms, and Imel et al. (2011), who discovered that therapists' variable effectiveness in treating cannabis abuse was a partial function of clients' ethnicity, we also found differential therapist effectiveness to be related to whether clients were REM or White individuals. In particular, we found that therapist trainees varied in their effectiveness at reducing psychological symptoms among clients (cf. Budge et al., 2013), and that this variability was partially due to clients' racial/ethnic heritage. In other words, some therapists worked with REM clients who changed more than other therapists. These findings were independent of the severity of clients' initial symptoms. Thus, as predicted, some therapists were more effective working with White clients than REM clients, and these therapists likely would benefit from training, education, and supervision to enhance their cultural competence. Other therapists were more effective working with REM clients than White clients and, assuming that they were not ineffective with White clients, might serve as useful role models from which their colleagues could learn. These differences among therapists are best thought of as relative rather than absolute. That is, although Therapist A may be more effective than Therapist B with REM clients, we did not establish a standard by

Table 2
Summary of Multilevel Models With Bayesian Estimation

	Model 1: Client R/E status fixed Coeff. (95% CI)	Model 2: Client R/E status random Coeff. (95% CI)
Fixed effects		
Intercept	50.47 (46.73, 54.22)***	48.91 (45.13, 52.10)***
Client R/E	-0.83 (-4.89, 3.22)	-0.50 (-3.21, 5.79)
Therapist R/E	-2.37 (-16.46, 9.17)	0.19 (-13.23, 11.12)
Pre-OQ	0.86 (0.77, 0.96)***	0.88 (0.77, 0.97)***
Random effects		
Intercept		
Within	241.38 (194.27, 296.69)***	235.45 (195.32, 287.99)***
Between	22.99 (3.38, 64.74)***	18.59 (3.54, 58.44)***
Client R/E	—	19.30 (1.55, 76.94)***

Note. Coefficients are the mode of the posterior distribution. CI = 95% credible interval. R/E = race/ethnicity; Coeff. = coefficient.

*** $p < .001$.

which Therapist A would be deemed effective or ineffective. Indeed, it would be possible for both therapists to be effective—though Therapist A more so, both to be ineffective—though Therapist A less so, or Therapist A to be effective and Therapist B ineffective.

On the whole, the data suggest that cultural competence can be distinguished from general therapist competence. This finding replicates the results of Imel et al. (2011) in their study of therapists' treatment of adolescent cannabis users, as well as Larrison et al.'s (2011) research that detected therapist effects as a function of client race in community mental health center therapists. Based on this collection of studies, then, there does seem to be unique variability in therapist effectiveness that is associated with the client's race or ethnicity. These findings would support the continued emphasis in many counseling and clinical psychology programs on multicultural training. Future research could profitably explore the comparable effectiveness of different models of multicultural training (e.g., stand-alone courses vs. a curriculum infused with a multicultural focus), as well as investigate the specific components of training (e.g., didactic, experiential, clinical) that promote cultural competence. It would also be useful for training purposes for future studies to identify which multicultural competencies (e.g., awareness, knowledge, skills) were related to client outcome.

In some ways, the data from this study were clinically encouraging. In the aggregate, REM clients experienced as much reduction in psychological symptoms as White clients and they ended therapy with similar levels of symptoms. Furthermore, some therapists demonstrated noteworthy effectiveness working with REM clients. For instance, one therapist worked with an REM client whose OQ-45 scores decreased from 65 at intake to 26 at termination and a second REM client whose scores decreased from 76 to 50; both clients experienced reliable change on the OQ-45 in that their scores improved more than 14 points and they ended treatment in the subclinical range. If studied in further detail, therapists such as this one might serve as models for studying culturally competent practice. On the other hand, some therapists clearly struggled with their REM clients. For example, one therapist saw three REM clients, all of whom reported more distress at termination than at intake; two of these clients deteriorated significantly during therapy, and the three clients terminated with high OQ-45 scores of 80, 96, and 106, respectively. This same therapist's two White clients neither improved nor deteriorated, showing an average increase of one point on the OQ-45 from intake to termination. This therapist, then, who was ineffective with White clients and harmful with REM clients, could be considered a prototypical "pseudoshrink" needing particular training and supervision working with REM clients.

Unfortunately, we can only speculate as to factors that might account for why some therapists were more effective than others at reducing symptoms among REM clients. The data set with which we worked did not contain variables that were likely to provide insight into this important question. As is so often true of research on therapist effects, we only had access to relatively superficial variables such as gender and type of training program that reveal little about the actual mechanisms of change in psychotherapy. It is much more difficult to study constructs that would yield more valuable information about working effectively with REM clients, such as establishing a relationship where one is perceived as

trustworthy and expert, or managing one's cultural countertransference reactions (Gelso & Mohr, 2001), or effectively communicating empathy about a culturally related topic when the therapist is White (Gaztambide, 2012; Hook, Davis, Owen, Worthington, & Utsey, 2013).

Although the data from the study were clinically encouraging in some ways, in other ways, they were clinically discouraging. Roughly two thirds of clients did not experience symptom reduction over the course of therapy. This finding could be due to (a) the brief nature of treatment (average number of sessions = 5.42); (b) a floor effect resulting from relatively low OQ-45 scores at the start of treatment (the mean pretreatment OQ-45 score for both White and REM clients was below 65, or in the subclinical range); and (c) the fact that treatment ended for many clients at the end of the semester, when many students experience stress due to final exams.

A major limitation to the present study is the relatively small number of clients per therapist, as well as the small number of therapists overall. Although this study contained nearly three times the number of therapists as Imel et al. (2011), the samples in both studies fall short of what might be considered ideal. Despite the small sample, effects were large enough to be detected. Future studies with more therapists and more clients per therapist will be needed to replicate findings from this study and to allow for the examination of multiple intersecting client cultural characteristics, such as race, sexual orientation, and religiosity. Another limitation to the present study is that all therapists were graduate student trainees, although previous research on therapist effects has found that therapist experience, age, and professional degree tend to account for little variability in client outcome (Tracey, Wampold, Lichtenberg, & Goodyear, 2014; Wampold & Brown, 2005). Despite these limitations, findings from this study served to replicate previous research that suggests that therapists do differ in their effectiveness with REM clients, both in the decrease in psychological symptoms their clients experience and how quickly that change takes place. The next steps in this line of research would be to replicate and extend these findings with a larger sample, preferably not from the same institution, a different outcome measure, and to include variables that provide insight into the characteristics and therapeutic behaviors of multiculturally expert therapists.

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Practice Review

This is an open invitation for authors to submit what Charlie Gelso developed and termed a *Practice Review* for possible publication in *Psychotherapy*. I want to continue this series as a step toward enhancing the value and relevance of scientific research on psychotherapy and related processes to practice.

The general aim of the *Practice Review* is to clarify, as much as the current state of knowledge permits, what empirically-derived findings in a given area imply for practice in that and related areas. In this type of review article, the reviewer begins the process with the intent of deriving implications for practice from the research and theory that is examined. Much like program evaluation research, the central question for the writer of a *Practice Review* may be phrased as: "Despite the near inevitability of at least somewhat mixed findings on virtually any topic, what is the most likely relationship between these variables, and what does that relationship imply for the practitioner?"

The above kind of question is based on an awareness that the practitioner must do his or her practice, despite the general lack of fully consistent research findings; and it will be useful in that practice if the *best available* knowledge were used. This, of course, is not to say that the reviewer may take a cavalier attitude toward drawing implications for practice. The reviewer needs to derive such implications with great care. At the same time, the *Practice Review* does not convey the same degree of scientific skepticism that is typical of the classical scholarly review. For example, in the traditional scholarly review, as in classical scholarly inquiry in general, one takes a very conservative attitude toward accepting results. Substantial evidence must accumulate before we may safely say a given finding is confirmed and valid. In the *Practice Review*, on the other hand, the investigator searches for the *most likely* conclusion, when all evidence is weighed, and then seeks to place that conclusion within the context of practice.

The process of relating a "most likely conclusion" or finding to practice is rarely if ever a straightforward or linear process. As but one example, the most likely conclusions about the role of duration of treatment in outcome is that, other things being equal, the longer the therapy (at least up to a certain point), the more positive the outcomes. What implications does this have for the practitioner? For implications to be drawn, this finding needs to be placed within the context of related findings, existing theory, and other factors (e.g., pragmatic ones) that help the practitioner conceptualize duration factors in his or her practice. Placing findings within contexts such as these may well modify the findings.

With these considerations in mind, the following guidelines are offered for those who write *Practice Reviews*:

1. Your set from the beginning should be to find out what are the most likely conclusions about the relationships under investigation.
2. In doing so, consider how particular findings may be integrated with related findings in your area of review.
3. Once the most likely conclusions are arrived at and placed in the context of related knowledge, discuss what these findings imply for the practitioner.
4. In relating findings to practice, show an appreciation of the likelihood that the findings-to-practice links will not be direct and clear cut. Rather, given findings ("facts") may relate to practice through their connection to theories, clinical wisdom, practical and political concerns, etc.
5. Although the refrain, "more research is needed," is virtually always valid, the practice review must not hide behind scientific equivocation. Rather, the approach ought to be that, although more research is surely needed, here is our best available knowledge and what it implies for practice.

Although the length of practice reviews should be dictated by the subject matter, such reviews generally should be limited to about 25 pages of text. Reviews of relatively narrow topics should naturally be much briefer. Authors are invited to contact me if they are considering writing such a review but have questions about the process. Email me at Psychotherapy@adelphi.edu.